July 2026 BRIEF

CMS TEAM · IMPACT REPORT

The Surgical Reform Decade is Here

Leveraging AI to meet the moment — strategy, frameworks, and procedure-level use cases for the TEAM era.



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INSIDE THE B

What you'll find in this report

A field guide for hospital and health-system leaders preparing for the first mandatory surgical episode model under CMS TEAM — and the wider wave of accountability that follows through 2030.

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BACKGROUND · LEVERAGING AI TO MEET THE MOMENT

The Surgical Reform Decade is Here



Older adults now drive ≈40% of U.S. inpatient surgical volume.



Multimorbidity is the norm for the modern older-adult surgical patient.

American hospitals are entering the most consequential decade of modern surgical care. By 2030, every Baby Boomer will be 65 or older, with nearly 71 million Americans eligible for Medicare.[1] Older adults already drive a disproportionate share of surgical volume that is expected to rise as the population ages. Currently, adults 65 and older account for nearly 40% of inpatient surgeries,[2] with 19.1% undergoing surgery within the last year.[3]

These are not surgical patients of a generation ago. Multimorbidity is now the norm, affecting roughly 74% of older emergency general surgery patients and more than 80% of those over the age of 85.[4] Frailty, dementia, polypharmacy, and social vulnerability are no longer exceptions, they define the modern older adult surgical population as a result. The urgency is demographic and systemic. These are the very populations and procedures CMS models target to not only control spending but to improve overall quality of care and the patient experience.

At the same time, the workforce is contracting. The AAMC projects a shortage of up to 121,000 physicians by 2030, including 20,000–30,000 surgeons,[5] while nursing shortages persist nationwide. Hospitals are being asked to deliver more complex care to a sicker population with fewer clinicians per bed.

This pressure coincides with a sharp compression of inpatient care. Average length of stay for joint replacement has declined from 3.0 days in 2012 to 1.1 days in 2023, and more than half of Medicare total knee arthroplasties are now performed on an outpatient basis.[6] The surgical episode has shifted beyond the hospital, yet expectations for accountability and care quality now extend across the entire continuum, from pre-operative care through discharge and into skilled nursing and home recovery.

References 1–6 · Full numbered list on page 12.

BACKGROUND · THE MANDATE WIDENS

From single episode to system-wide accountability

CMS programs such as TEAM formalize this shift, holding hospitals accountable for the full 30-day episode, where a substantial share of costs occurs in the post-acute phase.[7] Performance is assessed not only on total spending but also on quality measures, including readmissions and patient-reported outcomes. As a result, hospitals may remain within target spending yet still incur penalties if quality benchmarks are not met.[7]

As a result, health systems must robustly operationalize coordinated, continuous, and evidence-based care across the entire perioperative continuum, from preoperative optimization through recovery at home. This level of orchestration is not achievable with traditional manual approaches. AI-enabled platforms such as Rainfall Health are becoming essential, offering real-time visibility into patient progress, care processes, and projected outcomes, replacing delayed, retrospective reporting with actionable insight.

CMS TEAM is the first wave of mandated participation, with a total of 741 hospitals,[7] but it is not the only one. The CMS Age-Friendly Hospital Measure (IQR attestation began 2025, FY 2027 payment determination) and CJR-X (proposed to launch October 1, 2027 — a mandatory, nationwide 90-day joint replacement episode model with two-sided risk on day one)[8] extend the same accountability discipline to the ~2,500+ hospitals not required to participate in TEAM.

Within 24 months, virtually every IPPS hospital in the country will sit inside a mandatory surgical episode model. The infrastructure built for TEAM in 2026 is the same infrastructure that will define readiness for the rest of the decade.



Coordinated, continuous care across the perioperative continuum.

CMS TEAM is the first wave — within 24 months, virtually every IPPS hospital in the country will sit inside a mandatory surgical episode model.

≈40%
of U.S. inpatient surgeries are performed on adults 65+

19.1%
of adults 65+ had surgery in the past year

741
TEAM hospitals (2026) — then 2,500+ via CJR-X (2027)

$1.2M
Avg. projected loss per TEAM facility, year 1

References 7, 8, 10 · Full numbered list on page 12.

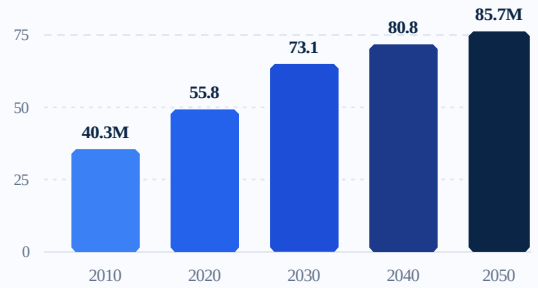
01 THE DEMOGRAPHIC WAVE

More older adults. More surgical episodes. Mandatory accountability.

Adults age 65+ are the fastest-growing segment of the U.S. population — and the fastest-growing segment of surgical demand.

Population 65+ more than doubles

Americans age 65+, in millions, 2010–2050



More than 1 in 5 Americans by 2030

Share of U.S. population that is age 65+

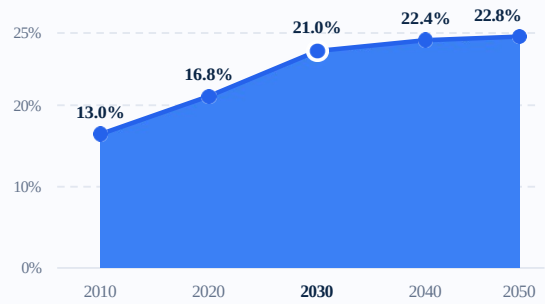


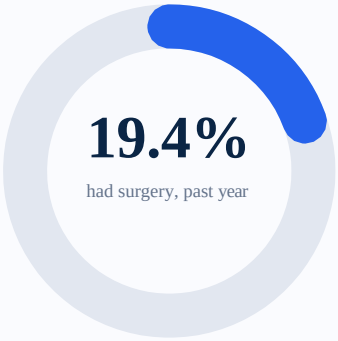
Figure 1. The 65+ population grows from 40.3M (2010) to 85.7M (2050), more than doubling, while the 65+ share of the U.S. population crosses 20% — more than 1 in 5 Americans — by 2030. [1]

In 1980 there were roughly 26 million Americans over 65. By 2030, that number will exceed 73 million, and by 2050, 86 million. The share of the U.S. population that is 65+ will cross 21% before this decade ends. Every major surgical specialty will see its case mix shift toward older and more complex adults.

02 SURGICAL DEMAND IN OLDER ADULTS

Why surgical reform targets older adults

Adults 65+ have the highest annual surgery prevalence and account for the majority of all five mandated TEAM procedures.



Surgery is concentrated in older adults

Nearly 1 in 5 adults age 65+ underwent surgery in the past year — the highest annual prevalence of any age group. Across CABG, joint replacement, spinal fusion, major bowel, and hip & femur fracture, older adults make up the majority of all five mandated TEAM procedures.

Had surgery (19.4%) No surgery (80.6%)

Older adults dominate the five TEAM procedures

Approximate share of each procedure performed on adults 65+

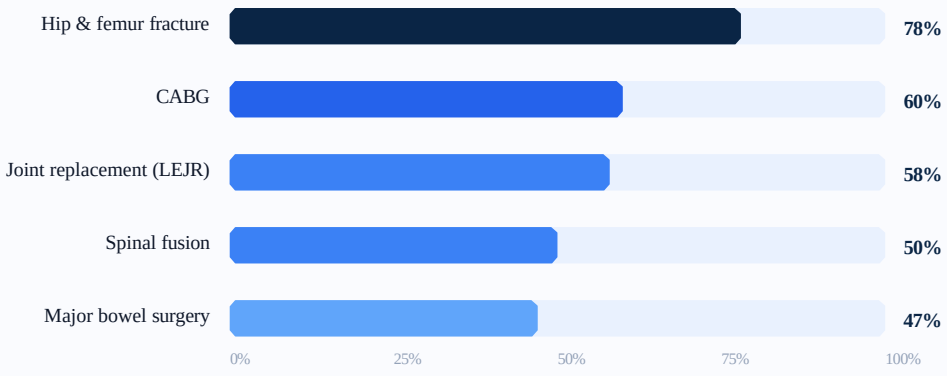


Figure 2. Surgical prevalence in adults 65+ and approximate share of each TEAM procedure performed on older adults. [3,9]

Hip and femur fracture treatment is overwhelmingly an older-adult procedure (~78%); CABG, joint replacement, spinal fusion, and major bowel surgery all draw much of their volume from the 65+ population. This is the operational reality TEAM was written for — procedures with the highest age skew, highest variation in post-acute spending, and highest readmission rates.

References 3, 7, 9 · Full numbered list on page 12. Procedure shares approximate.

03 THE TEAM MANDATE

The TEAM Mandate: scale and stakes

741 mandated hospitals, 204k episodes annually, and up to 20% of episode revenue at risk by PY3.

741
TEAM-mandated hospitals

204,014
Annual surgical episodes

$1.2M
Avg. projected loss per facility

56%
Hospitals projected to lose money

Per-procedure economic swing under TEAM

Max win / loss per case ($). Track 1 / Track 3 range.



Figure 3. TEAM scope and per-procedure economic swing. Per-case wins/losses model the full Track 1 / Track 3 range. [7,10]

TEAM is not a pilot. As of January 1, 2026, 741 acute-care hospitals across 188 CBSAs will be reconciled annually on the cost and quality of every CABG, major bowel, lower extremity joint replacement, surgical hip and femur fracture, and spinal fusion episode across a 30-day post-discharge window.[7] Industry modeling estimates 56% of mandatory participants will lose money in the first reconciliation cycle, with an average loss of $1.2M per facility.[10] For systems performing all five procedures at scale, total exposure across the five-year performance window can exceed $14M before HRRP, HAC Reduction, and VBP penalties are layered in.[10]

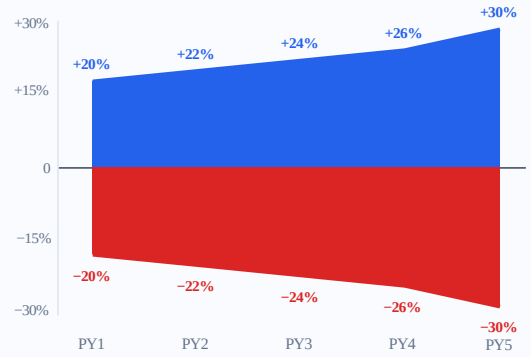
04 PENALTIES & INCENTIVES — THE RISK ENVELOPE

Up to ±30% of cumulative target spend on the line by PY5

TEAM is two-sided. The same five-year window that exposes hospitals to multimillion-dollar penalties also opens an equal-and-opposite incentive payment opportunity for high performers.

Track 3 stop-gain / stop-loss envelope by year

Bands by performance year



5-yr exposure: mid-volume Track 3 hospital

Cumulative reconciliation impact ($)

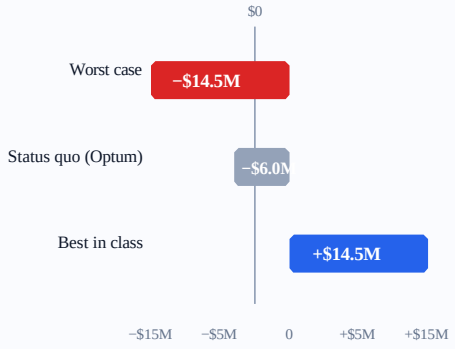


Figure 4. Track 3 stop-gain / stop-loss bands by performance year, plus modeled cumulative 5-year reconciliation impact for a mid-volume hospital — spanning −$14.5M (worst case) to +$14.5M (best in class), with the Optum status-quo projection at about −$6M. [7,10]

Under Track 3 (the default for most TEAM hospitals after PY1), reconciliation is capped at ±20% of cumulative target spend, then adjusted up to an additional ±10% based on the Composite Quality Score.[7] By PY5, the spread between a worst-case and best-in-class performer at the same hospital can approach 60 percentage points of episode revenue. The math is symmetric, but the discipline required to land on the upside is not.

> Real-time tracking of process steps, post-acute placement, and quality-measure adherence is what separates the two outcomes.

05 PROVEN RESULTS — CALIFORNIA

What AI-enabled TEAM operations look like in practice

Case examples from California — the largest concentration of TEAM-mandated hospitals in the country.

CALIFORNIA — STATEWIDE

110 mandated hospitals across 26 counties

California has more TEAM-mandated facilities than any other state.[11] Greater LA alone hosts 59 facilities; the Bay Area 22; Central Valley 17; San Diego 8; Northern California 8; Central Coast 4. Every one of them must operate the same five-procedure episode model as of January 1, 2026.[7,11]

110	26	59	5 yr
TEAM-mandated facilities	California counties	Facilities in Greater LA	Mandatory performance period



TEAM-mandated facilities by California region

Greater Los Angeles	59
Bay Area	22
Central Valley	17
San Diego	8
Northern California	8
Central Coast	4

Bubble area is proportional to the number of TEAM-mandated facilities in each region. Greater Los Angeles alone accounts for more than half the state's mandated volume.

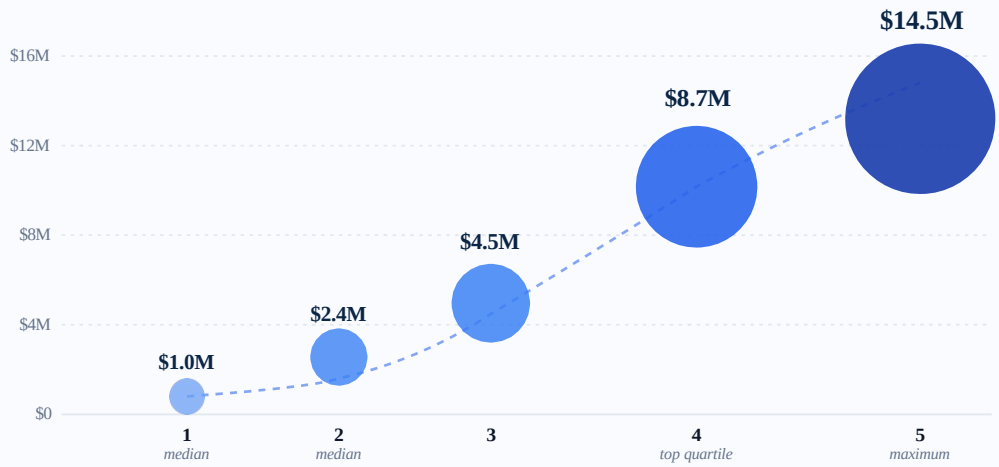
06 CALIFORNIA — EPISODE ECONOMICS

$2.4M median, $14.5M peak five-year impact

A Rainfall Health analysis of 17 California health systems modeled five-year financial exposure under TEAM as a function of how many of the five mandated procedures each system performs at meaningful volume.[12]

> Impact scales nonlinearly: systems performing 4–5 procedures (CABG, major bowel, LEJR, SHFFT, spinal fusion) face exponentially higher risk than single-procedure systems.

Projected 5-year financial impact by number of TEAM procedures performed at volume



$2.4M	$8.7M	$14.5M	17
Median 5-year impact	Top quartile impact	Maximum (5-procedure systems)	California systems analyzed

RAINFALL HEALTH · PLATFORM RESULTS

Up to 20% Medicare reimbursement impact, $85M new revenue over 5 years

San Francisco-based Rainfall Health (CEO Eddie Qureshi; $15M Series A led by Two Bear Capital, February 2026)[13] built an AI-enabled compliance and reimbursement platform specifically for the TEAM mandate. Its capabilities span pre-surgical CDI and PROMs capture, intra-episode cost tracking, post-acute provider matching, and the RAIN Compliant™ verifiable-readiness designation.[12]

20%	$85M	70–80%	5
Medicare reimbursement impact (up to)	New Medicare revenue over 5 yrs	Readmission ID rate	TEAM procedures end-to-end

WHAT THIS MEANS FOR YOU

The window is 2026. The discipline you build for TEAM is the discipline that will define the next decade of surgical care.

AI-enabled TEAM operations are not a revenue-cycle add-on — they are a clinical-AI strategy leveraged to reduce administrative burden while optimizing surgical care delivery in a vulnerable patient population. Documentation, care pathway adherence, post-acute placement, referral coordination, and the integration of evidence-based practices are all critical points where an AI-enabled platform such as Rainfall Health can connect care team members and augment performance in real time.

The C-suite question is no longer whether to deploy an AI platform as part of TEAM reporting, but whether to deploy it before or after the first reconciliation cycle, once penalties are realized. Given the compounding-penalty math, the possibility for incentive payments, and the procedure-mix asymmetry shown above, the cost of waiting is materially higher than the cost of acting proactively.

REFERENCES & SOURCES

References

Numbered citations correspond to in-text superscripts throughout this brief. Government and peer-reviewed sources are listed first; industry modeling and internal Rainfall Health analyses follow.

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2 Becher RD, Vander Wyk B, Leo-Summers L, Desai MM, Gill TM. *Trends in the use of major and ambulatory surgery in older adults in the United States.* JAMA Surg. 2023;158(11):1175–1183.

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7 Centers for Medicare & Medicaid Services. *Transforming Episode Accountability Model (TEAM) — Final Rule (CMS-5540-F).* Federal Register. August 28, 2024.

8 Centers for Medicare & Medicaid Services. *FY 2027 IPPS Proposed Rule — CJR-X (Comprehensive Care for Joint Replacement Expansion).* April 10, 2026.

9 American College of Surgeons. *ACS NSQIP & Medicare claims procedure-mix data: age distribution of CABG, LEJR, SHFFT, spinal fusion, and major bowel surgery.* 2023–2024.

10 Advisory Board / Optum. *TEAM Financial Impact Model: First-cycle hospital exposure analysis.* 2025.

11 Rainfall Health Policy & Research. *California TEAM-Mandated Hospitals Brief.* San Francisco, CA: Rainfall Health; April 2026.

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All figures and analyses in this report were independently compiled by Christina Keny, PhD, MHA, RN, CPHQ for Rainfall Health. Procedure-share estimates are approximate and intended for strategic planning, not for individual clinical decision-making.